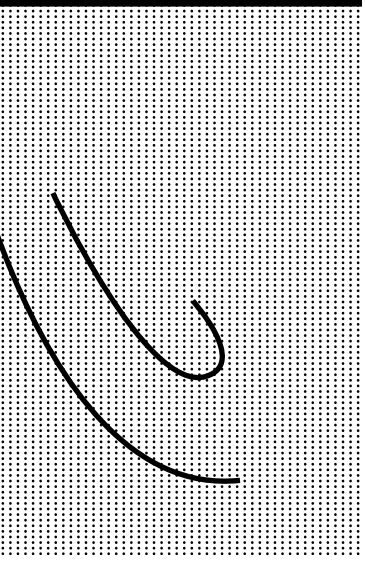


**INVESTMENTS IN PRIMARY HEALTHCARE,** INCLUDING TRAINING THE WORKFORCE, AND **USING DIGITAL TOOLS** WILL BE KEY

**The COVID pandemic posed an unexpected and** unprecedented challenge for humanity and forcefully brought home the point that good health is central to economic well-being—for countries and for the world as a whole. Not only is it the biggest economic shock of the past two centuries, with the global economy losing over $8 trillion, but poorer countries and people have borne the brunt, leading to an even more sharply unequal world. Further, it is expected that this generation of students impacted by large-scale school closures could lose $17 trillion in lifetime earnings. COVID also showed us that the returns on investments in science and technology are huge, with the knowledge and tools to conquer the effects of the virus coming from science. It underlined the need for countries to invest in public health and primary healthcare, especially in the workforce and essential commodities, like oxygen. It unfortunately also exposed the lack of global solidarity and the role of geopolitics in responding to a global threat, with nationalism and shortsighted policies leading to vast inequities in access to countermeasures across populations. Further, because it is the first pandemic in the social media age, the resulting 'infodemic' meant that people were exposed to huge amounts of information, which included a lot of misinformation and disinformation, which, in turn, served to confuse and mislead them on the various measures of protection like masking and vaccination.

As the world slowly emerges from the acute phase of the pandemic, there are still some uncertainties regarding any new variants of concern, but we have a lot of tools to fight the virus now, and must look ahead to face our major public health challenges and create a healthy population. Today, the major causes of premature mortality (death before 70 years of age) in India are non-communicable diseases or NCDs (heart disease, diabetes, stroke, cancer and chronic respiratory disease). Prevention plays a major role in NCDs and this means attention to the risk factors—diet, physical activity, air pollution, tobacco, alcohol, etc. as well as early detection through screening, better diagnostics and affordable treatment. To have an impact on reducing the disease burden, we will need the whole-of-government approach, with the private sector and civil society also playing important roles. Increasingly, commercial determinants of health (which can be positive or negative) need policy action—good examples are the banning of trans fats in food, tobacco regulation and taxes, incentivising the consumption of millets by making them available through the public distribution system (PDS), and regulating the sale of high-fat, high-salt and high-sugar food and beverages in schools.

Science and health literacy are going to be key factors for an enlightened and healthy population. This must start in schools, but people of all ages need credible and high-quality information presented to them in easily understandable ways. Maintaining good health often requires behavioural change, social support and incentives, our public health cadres must include social scientists, behavioural economists and anthropologists, and communication strategies need to be tailored to social and cultural contexts. Given the wide reach of social media, government public health departments should have a group that deals with 'infodemics' and that is skilled in anticipating, monitoring and responding to circulating misinformation (often called social listening). The need for a separate public health cadre in each state and at the Centre cannot be overemphasised. The difference between clinicians who treat sick people and a public health cadre (including doctors, epidemiologists, nurses, nutritionists, entomologists,